

Office of Enforcement and Removal Operations
ICE Health Service Corps

U.S. Department of Homeland Security
500 12th Street, SW
Washington, D.C. 20536



U.S. Immigration
and Customs
Enforcement

August 28, 2023

MEMORANDUM FOR:

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Assistant Director
ICE Health Service Corps

(b)(6),(b)(7)(C),(b)(7)(F)

THROUGH:

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FROM:

CAPT (b)(6),(b)(7)(C),(b)(7)(F) RN
Investigator
ICE Health Service Corps

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SUBJECT:

Mortality Review – Report of Findings
Ernesto ROCHA-Cuadra, (b)(6),(b)(7)(C)

Executive Summary:

On June 23, 2023, U.S. Immigration and Customs Enforcement (ICE) Health Service Corps (IHSC) received notification of the death of ICE detained noncitizen Ernesto ROCHA-Cuadra, (b)(6),(b)(7)(C) ICE assigned Mr. ROCHA-Cuadra, a 42-year-old Nicaraguan male, to Central Louisiana ICE Processing Center (CLIPC), in Jena, Louisiana (LA), from May 19, 2022 to June 23, 2023, the date of his death. Mr. ROCHA-Cuadra's autopsy is pending.

Prior to ICE custody, on July 29, 1992, the former U.S. Immigration and Naturalization Service admitted Mr. ROCHA-Cuadra to the U.S., as a lawful permanent resident. On February 27, 2009, ICE Enforcement Removal Operations (ERO) encountered Mr. ROCHA-Cuadra,

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determined him removable from the U.S. pursuant to Section 237(a)(2)(A)(ii) of the Immigration and Nationality Act (INA), and later removed Mr. ROCHA-Cuadra from the U.S. to Nicaragua.

On April 17, 2022, the U.S. Border Patrol (USBP) encountered Mr. ROCHA-Cuadra in the U.S. (location unknown) and determined him removable under section 241(a)(5) of the INA. On April 26, 2022, USBP transferred Mr. ROCHA-Cuadra to ERO New Orleans, and ERO New Orleans transferred Mr. ROCHA-Cuadra to Richwood Correction Center (RCC), in Richwood, LA. On May 19, 2022, ERO New Orleans transferred Mr. ROCHA-Cuadra to CLIPC, where he remained until June 23, 2023.

On May 19, 2022, during his intake screening, Mr. ROCHA-Cuadra reported a history of insomnia and anxiety related to his detention, chronic right (R) wrist pain, back pain, and left knee pain. A registered nurse (RN) completed Mr. ROCHA-Cuadra's intake screening and cleared him for general population.

Between May 19, 2022 and June 22, 2023, the CLIPC medical staff provided Mr. ROCHA-Cuadra treatment for his preexisting conditions and his new diagnosis of hypertension (high blood pressure) and hyperlipidemia (elevated cholesterol and triglyceride levels).

On June 23, 2023, at approximately 12:27 p.m., Mr. ROCHA-Cuadra experienced syncope (loss of consciousness), and a CLIPC correctional officer announced the emergency via facility radio. Medical staff responded to the emergency, and at approximately 12:31 p.m., CLIPC's correctional staff activated emergency medical services (EMS). The medical and correctional staff then transported Mr. ROCHA-Cuadra to the medical clinic for further evaluation and treatment, during which time he became responsive to verbal and noxious stimuli (stimuli that threaten the body's tissues and are liable to cause pain). EMS personnel arrived and transported Mr. ROCHA-Cuadra, while still conscious, to Lasalle General Hospital (LGH) emergency department (ED), in Jena, LA, where a physician evaluated him with a diagnosis of status post syncope. At approximately 1:15 p.m., Mr. ROCHA-Cuadra became unconscious, followed by respiratory arrest, and subsequently he experienced cardiac arrest.

On June 23, 2023, at 2:59 p.m., an LGH physician declared Mr. ROCHA-Cuadra deceased.

Mortality Finding:

As a result of Mr. ROCHA-Cuadra's death, the IHSC Assistant Director requested a mortality review to learn from his death by reviewing the care provided and the circumstances leading up to his death. The goal of the mortality review is to determine the appropriateness of clinical care; ascertain whether changes to policies, procedures or practices are warranted; and identify issues requiring further study.

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A mortality review committee (MRC) determined that Mr. ROCHA-Cuadra's medical care at CLIPC was provided within the safe limits of practice; however, the MRC determined the following: 1) CLIPC correctional staff who initially responded to Mr. ROCHA-Cuadra's emergency had a limited sense of urgency; 2) Noncitizens reported Mr. ROCHA-Cuadra had access to and used illicit drugs while at CLIPC.

The MRC convened on August 18, 2023, and consisted of the following IHSC members: Clinical Director (b)(6),(b)(7)(C),(b)(7)(F) MD, physician reviewer; Medical Asset Support Team (MAST) Advanced Practice Provider (APP) Commander (CDR) (b)(6),(b)(7)(C),(b)(7)(F) APP reviewer; MAST Senior Nurse Specialist Lieutenant (b)(6),(b)(7)(C),(b)(7)(F) nurse reviewer; and Risk Manager Lieutenant Commander (b)(6),(b)(7)(C),(b)(7)(F) risk management reviewer.

The following summary of events was based on: 1) Mr. ROCHA-Cuadra's CLIPC medical records and EMS report; 2) incident and notification reports; 3) ICE ENFORCE Alien Removal Module (EARM) and ICE ENFORCE Alien Detention Module (EADM) database records; 4) Mr. ROCHA-Cuadra's CLIPC detention file; 5) video footage from June 23, 2023; 6) an on-site review and staff interviews conducted by Fact Finders Captain (CAPT) (b)(6),(b)(7)(C),(b)(7)(F) and CAPT (b)(6),(b)(7)(C),(b)(7)(F) at CLIPC on August 2 to 4, 2023; and 7) applicable CLIPC policies and procedures, and ICE Detention Standards.

Summary of Events

On May 19, 2022, a registered nurse (RN) reviewed Mr. ROCHA-Cuadra's RCC transfer summary which reflected his diagnoses of right hand injury and acute reaction to stress, and the following medications: amitriptyline (antidepressant), hydroxyzine (anxiety), duloxetine (antidepressant), and fluticasone propionate nasal suspension (steroid treatment for allergies). The RN completed Mr. ROCHA-Cuadra's intake screening and documented his self-reported history of insomnia and anxiety since his detention, R wrist pain, left knee pain, and chronic lower back pain. Mr. ROCHA-Cuadra did not report any history of hypertension (HTN). The RN documented his blood pressure (BP) reading of 128/89 millimeters of mercury [(mmHg); normal: 120/80 mmHg], height of 5 feet, 6 inches, and weight of 180 pounds (lbs.). The RN completed Mr. Rocha-Cuadra's coronavirus disease 2019 test, which showed negative results, submitted a referral to a behavioral health provider (BHP) for further evaluation, created an auto-generated advanced practice provider (APP) appointment for the next day, and cleared Mr. ROCHA-Cuadra for placement into general population.

On May 20, 2022, a BHP evaluated Mr. ROCHA-Cuadra for his reported history of insomnia and anxiety, diagnosed him with post-traumatic stress disorder (PTSD), and scheduled him a follow-up appointment in two weeks.

- On the same day, an APP completed Mr. ROCHA-Cuadra's initial physical examination (PE) and documented his BP reading of 130/75 mmHg. Mr. ROCHA-Cuadra reported a

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history of R hand injury with deformity, chronic left knee pain, elevated BP without a diagnosis of HTN, allergic rhinitis, constipation, acute stress reaction, and insomnia. The APP diagnosed Mr. ROCHA-Cuadra with a R hand contusion, R palmar fascial fibromatosis (a deformity of the right hand), acute stress reaction, elevated BP reading without a diagnosis of HTN, allergic rhinitis, and constipation. The APP ordered baseline laboratory tests, which showed values consistent with hyperlipidemia (elevated cholesterol and triglyceride levels), and an electrocardiogram (a recording of the heart's electrical activity), which showed a normal sinus rhythm. The APP ordered Mr. ROCHA-Cuadra acetaminophen (analgesic) 325 milligram (mg), two tablets, by mouth (PO), four times a day, as needed (PRN), menthol-methyl salicylate cream (analgesic), apply as directed, externally, at bedtime, PRN, amitriptyline 25 mg tablet, PO, PRN, hydroxyzine 50 mg, two capsules, PO, three times a day, PRN, duloxetine 60 mg, one capsule, PO, daily, fluticasone propionate nasal suspension, two sprays to each nostril, daily, cetirizine (anti-histamine) 10 mg, one tablet, PO, daily, docusate sodium (stool softener) 100 mg, one capsule, PO, daily, PRN, and daily BP checks for three days, completed a referral for an off-site orthopedic appointment for his R hand deformity, and scheduled him a follow-up appointment.

On May 23, 2022, a licensed practical nurse (LPN) documented Mr. ROCHA-Cuadra's BP reading of 122/82 mmHg.

Between May 24 and October 23, 2022, CLIPC completed Mr. ROCHA-Cuadra BP checks 15 times, and five of his BP readings showed abnormal results (95/88, 142/86, 148/88, 134/93, and 158/88 mmHg).

On October 24, 2022, an APP completed a chronic care (CC) follow up for Mr. ROCHA-Cuadra's R hand pain. The APP reviewed previous laboratory results, which showed values consistent with prediabetes and hyperlipidemia, diagnosed Mr. ROCHA-Cuadra with hypertriglyceridemia (elevated triglyceride level) and hyperprolactinemia (elevated prolactin hormone level), documented a BP reading of 134/93 mm Hg and a weight of 236 lbs., ordered gemfibrozil (lipid lowering agent) 600 mg, one tablet, PO, twice a day before morning and evening meals, and scheduled a follow-up appointment in six weeks.

Between October 25, 2022 and January 16, 2023, CLIPC medical staff completed Mr. ROCHA-Cuadra's vital signs (VS) eleven times, and two BPs showed abnormal results (139/93 and 149/113 mmHg).

On January 24, 2023, an APP evaluated Mr. ROCHA-Cuadra's chronic R hand pain, documented a BP reading of 104/75 mmHg, ordered ibuprofen (non-steroidal anti-inflammatory) 800 mg, one tablet with food, PO, three times per day, PRN, and a follow-up at next primary care appointment.

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On February 17, 2023, an APP completed Mr. ROCHA-Cuadra's CC follow-up appointment, reviewed previous laboratory results, which showed values consistent with prediabetes and hyperlipidemia, documented a BP reading of 128/78 mmHg, diagnosed Mr. ROCHA-Cuadra with essential HTN, and ordered lisinopril (antihypertensive) 20 mg, one tablet, PO, daily, and atorvastatin (lipid lowering agent) 40 mg, one tablet, PO, daily.

On March 22, 2023, an APP completed Mr. ROCHA-Cuadra's CC follow-up appointment, documented an elevated BP reading of 144/81 mmHg, increased his lisinopril medication from 20 mg to 40 mg, one tablet, PO, daily, and scheduled a follow-up appointment in three weeks.

On April 12 and May 9, 2023, an APP completed Mr. ROCHA-Cuadra's annual PE and CC respectively, reviewed previous laboratory results, which showed values consistent with prediabetes and hyperlipidemia, documented BP readings of 122/88 mmHg and 113/76 mmHg, weight of 255 lbs., and ordered a follow-up appointment within four weeks.

On June 12, 2023, an APP completed Mr. ROCHA-Cuadra's follow-up CC appointment. Mr. ROCHA-Cuadra reported having elevated BP when he experiences increased pain. The APP documented a pain level of 5/10 on a 1-10 pain scale, an elevated BP reading of 164/86 mmHg, ordered diclofenac sodium gel (analgesic) 1%, one application, externally, twice daily, encouraged weight loss due to his weight of 263 lbs., and scheduled a follow-up appointment in four weeks.

On June 22 and 23, 2023, Mr. ROCHA-Cuadra inhaled an unknown substance. Based on eyewitness' accounts, Mr. ROCHA-Cuadra utilized radio batteries to ignite and inhale the unknown substance.

On June 23, 2023, at approximately 12:33 p.m., an RN responded to a medical emergency for medical assistance, located in Eagle Bravo dorm. Upon arrival to the dorm, the RN observed Mr. ROCHA-Cuadra sitting in a chair, exhibiting shallow breaths, responding only to painful stimuli, and opening and closing his eyes. The RN placed Mr. ROCHA-Cuadra on a gurney, transferred him to the medical clinic, and documented Mr. ROCHA-Cuadra became more responsive to verbal stimuli during transport to the medical clinic.

At approximately 12:36 p.m., an APP evaluated Mr. ROCHA-Cuadra for unresponsiveness and documented his initial VS: BP reading of 76/43 mmHg, Pulse (P) of 68 bpm, and oxygen saturation level of 65 percent, [(%); normal: 95-100%]. The APP ordered supplemental oxygen therapy at 8 liters per minute, administered naloxone HCL liquid (a medication used to help reverse opioid overdose) 4 mg/0.1 milliliters, intranasally, single dose; an electrocardiogram, which showed sinus tachycardia (increased heart rate), and a P of 103 bpm. The APP documented Mr. ROCHA-Cuadra became more responsive after treatment. Mr. ROCHA-Cuadra's BP reading increased to 94/57 mmHg, and his oxygen saturation level increased to 80 -

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97%. The APP referred him to LGH ED via emergency medical services (EMS) EMS, in Jena, LA, for further evaluation.

At approximately 12:40 p.m., EMS personnel arrived and transferred Mr. ROCHA-Cuadra to LGH for further evaluation.

At approximately 12:55 p.m., EMS personnel transferred Mr. ROCHA-Cuadra to LGH, and the ED staff assumed his care and observed him in a stable condition and responding appropriately to questions.

At approximately 1:11 p.m., Mr. ROCHA-Cuadra suffered a syncopal episode, followed by respiratory arrest, and he subsequently went into cardiac arrest. LGH medical staff initiated life-saving efforts, but Mr. ROCHA-Cuadra did not regain consciousness.

At 2:59 p.m., an LGH physician declared Mr. ROCHA-Cuadra deceased, with a preliminary cause of death as cardiac arrest.

The final autopsy report is still pending.

ICE detention standards used for this review: Performance-Based National Detention Standards (PBNDS), 2011 with 2016 Revisions, and the National Commission on Correctional Health Care (NCCHC) Standards for Health Services in Jails, 2018. The following are identified program strengths, weaknesses, and recommendations found during this review.

Strengths

Strengths identified during this investigation were: 1) All staff who responded are CPR certified. 2) Medical staff responded within four minutes of the medical emergency announcement. 3) The responding staff effectively communicated the need to call 911. 4) The staff transported Mr. ROCHA-Cuadra to the urgent care clinic in a timely manner. 5) Medical and correctional staff demonstrated cohesive communication and interaction. 6) The responding staff were offered employee assistance program services after the medical emergency.

Weaknesses

1. Emergency Response:

- The correctional staff who initially responded to Mr. ROCHA-Cuadra's medical emergency responded with a limited sense of urgency.
- The responding correctional staff did not immediately recognize the need to properly check for airway, breathing, and circulation because they saw Mr. ROCHA-Cuadra "snoring."
- The responding staff did not participate in a debriefing after the emergency response.

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Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 4.3, Medical Care; section T. *Emergency Medical Services and First Aid.*
- NCCHC: 2018 Standards: J-C-04, *Health Training for Correctional Officers.*
- PBNDS 2011 with 2016 Revisions: Part 1.1, Emergency Plans; Section C. *Post-Emergency Procedures.*

2. Contraband:

- During interviews, other noncitizens reported Mr. ROCHA-Cuadra utilized an illegal unknown substance on June 22 and 23, 2023, and he utilized radio batteries to ignite and inhale the unknown substance.
- Noncitizens assigned to the special management unit have access to other noncitizens' cells, and correctional staff did not have consistent awareness of the policy.

Applicable standards of care for this finding:

- PBNDS 2011 with 2016 Revisions: Part 2.3, Contraband; section V. *Expected Practices.*
- PBNDS 2011 with 2016 Revisions: Part 2.12, Special Management Units; Section 2. *Expected outcomes.*

Recommendations

- Forward these findings to the IHSC Deputy Assistant Director of Health Care Compliance (DAD-HCC).
- The IHSC DAD of HCC will share these findings through appropriate communication channels to ICE, CLIPC's administrator and health authority, for review and any action(s) deemed appropriate.
- The respective IHSC HCC unit and ICE will ensure the corrective action plan, if applicable, is implemented and sustained.

End of report.